

LOCALISED ITCHY RASH

Check if the patient needs urgent attention → 67.

- If rash on scalp → 80.
- If very itchy, small red bumps and burrows in webspaces of fingers, axillae, waist or genitals, **scabies** likely → 69.
- If patches of dry, scaly, itchy skin on wrists, ankles, inside elbows or behind knees, **eczema** likely → 69.

Are there red itchy bumps that may have blistered or healed with darkening of skin?

Yes

Usually occurs in crops.



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Insect bites likely

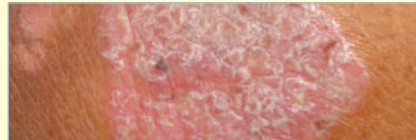
- Advise to reduce exposure to insects:
 - Treat pets, use bed nets, wash bedding, use insect repellents.
 - Clear away puddles of water around house.
- Advise to avoid scratching.
- Apply **calamine lotion** as needed.
- If severe itch, give **chlorphenamine** 4mg at night, or up to 6-8 hourly for up to 5 days.
- If blisters/heals with darkened skin, manage as likely **papular urticaria** → 22.
- If yellow crusts, **impetigo** likely → 78.

No: check site of rash.

Head/face, trunk or limbs

Ask where rash started and how it has progressed. Look at distribution of rash, check for raised edges and check nails.

Well-defined, raised plaques covered with silvery scale. Often on knees, elbows, lower back, scalp. May have pitted nails.

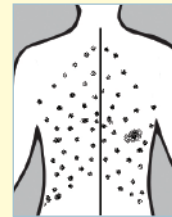


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Psoriasis likely

- Refer to specialist to confirm diagnosis.
- While waiting for appointment:
 - Moisturise skin with **emulsifying ointment (UE)** twice a day.
 - Apply **betamethasone 0.1%** ointment twice a day. Once improving, apply instead **hydrocortisone 1%** twice a day, then reduce to once a day. Stop as soon as better or
 - Apply **liquor picis carbonis (LPC) BP 5%** ointment once a day.
- Encourage to expose skin to sunlight before 10am or after 3pm for up to 30 minutes per day.

Started as one large ring on chest or back (herald patch) with fine scale in centre. Typically followed within 2 weeks by smaller, oval, scaly patches. May be in pattern of christmas tree on the back.

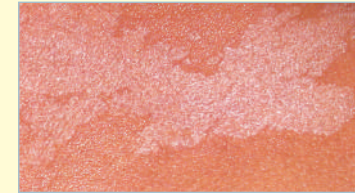


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Pityriasis rosea likely

- Reassure that rash will resolve within 2 months.
- Apply **aqueous cream (UEA)** 3 times a day.
- For itch:
 - Give **chlorphenamine** 4mg at night.
 - If itch no better or severe daytime itch, give instead **certirizine** 10mg daily.

Slow-growing lesion/s with raised edges/ring of scale, clear in centre



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Tinea corporis (ringworm) likely

- Advise to keep skin clean, to dry well and avoid sharing towels, clothes, combs and hair brushes.
- If on feet, encourage open shoes and avoid socks of synthetic material.
- Apply **clotrimazole 1%** cream 3 times a day or, if on feet, twice a day. Continue for 2 weeks after rash has cleared (at least 4 weeks for tinea pedis).
- If extensive or recurrent, test for HIV → 110 and diabetes → 17.
- If involves nails → 82.
- If extensive or no better after 1 month, refer.

Feet

Cracks, peeling or scaly lesions between toes, or thickened scaly skin on soles, heels and sides of feet.



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Tinea pedis (athlete's foot) likely

If diagnosis uncertain, discuss/refer.